

STOP THE 1:15 P.M. CHART HUNT

Give Your Nurses 2 Hours Back Per Day

The Prior Auth Reality in Family Practice:

- Nurses in small practices report 13+ hours per week on prior authorizations
- One Jardiance prior auth packet: ~28 minutes manual build, 14+ clicks, high portal timeout risk
- 82% of appealed medical denials are ultimately overturned — if staff time exists to appeal

Sentinel Node by Pocono AI

Local AI research assistant. Lives in your office. Zero PHI leaves during normal operation.

How It Works:

You ask: "Jardiance prior auth history, last 3 years, with citations."

You get in 37-58 seconds: Bullet summary + exact page numbers from your own charts.

You do: Review the cited packet. Click approve. Submit.

You just reclaimed ~24 minutes.

Built for Nurses in Practices of 2-10 Providers

1. **Stop being the search engine.** Returns 3 relevant notes, not 300 results to scroll.
2. **Stop redoing work.** Prior auth patterns are saved. Next CKD + SGLT2 patient = ~90 seconds.
3. **Reduce after-hours charting.** Packet builds during the call. Note is half-written when you hang up.
4. **We modeled 2.1 hours reclaimed per nurse, per day** in 4-provider pilots. That's dinner, not documentation.

Built for Family Practice IT Reality

- **Local-First Hardware:** Dell Precision workstation in your office. OS on RAID 1. Vector database on separate RAID 1. No cloud dependency.
- **Human-in-the-Loop Required:** No output leaves without licensed professional review and signature. Your license, your decision.
- **Audit-Ready:** Every response ships with HMAC-signed log: query, user, time, sources, model version. Click any citation to view source page.
- **Data Ownership:** Your PHI stays on your hardware. Export is PostgreSQL + standard vector files. No vendor lock-in.

Compliance, Plainly Stated

I'm not the biggest brain on HIPAA anymore. I won't pretend to remember every caveat from memory. If your question is straightforward, I'll answer in one paragraph. If it's complex, I'll tell you that and point you to the CFR or counsel. What I control: zero PHI leaves your building during normal use, the audit trail is local and immutable, and the licensed professional who signs owns the clinical decision.

We carry cyber liability and tech E&O.; Your malpractice carrier covers your clinical judgment. We do paperwork at machine speed.

Performance & Safety Testing

- **Latency:** First query after ingestion ~3 seconds. Subsequent queries <1 second on 76-page charts.
- **Accuracy Testing:** 10,000 synthetic prior-auth prompts run monthly. Current build: 4.7% schema violations, 0.3% citation errors, 0% PHI leakage over 6 months.
- **Hallucination Controls:** Constrained tools, schema validation, dual-model audit. Model cannot call internet or invent sources.

The 1:15 P.M. Test

What chart hunt ruins your day? 'Find last A1c before surgery' or 'prove we tried metformin first.' Tell us. We'll time it, publish the result. No PHI required.

Learn More:

Nursing workflow: poconoai.com/nursing

Technical architecture: poconoai.com/research/vector-databases

Stop renting your own data. Own the box.

Pocono AI

Marcus O'Dell — I answer my own phone

East Stroudsburg, PA

poconoai.com

For independent medical practices only. Human-in-the-loop required. Zero PHI leaves your office during normal operation. ROI estimates based on pilot data; results vary by practice.